

**Cannock Chase Hospital Respiratory Physiology Service
Community Diagnostic Centre**

BREATHLESSNESS SERVICE REFERRAL FORM

**Referral form requesting secondary care assessment of patients with chronic undiagnosed
breathlessness (over 8 weeks)**

**Patients with a known respiratory or cardiac condition where there is concern regarding worsening
of that condition should be referred directly to the relevant sub-speciality**

PATIENT DETAILS:

Name: Full Name(inc. middle)	Date of Birth: Date of Birth
Address: Home Full Address (single line)	Home Phone No: Patient Home Telephone
NHS Number: NHS Number	Mobile Phone No: Patient Mobile Telephone

GP DETAILS:

Name: Usual GP Full Name	Referral date: Long date letter merged
Address: Organisation Name Organisation Full Address (single line)	GP Signature: Free Text Prompt
Phone No: Organisation Telephone Number	GP email address: Organisation E-mail Address
	This is a requirement to enable the service to liaise with the GP if needed.

Please note: By completing this referral, and where clinically appropriate, the bronchodilator listed below will be delivered as per Lung Function protocols during the completion of the initial diagnostic bundle.

- 400mcgs Salbutamol via MDI and spacer device.

By signing this referral, you are agreeing for this medication to be administered to the patient, unless contraindicated.

ASSISTANCE:

Interpreter Required Yes ☐ No ☐	If yes, main spoken language is:	Need for advocate/carer present? Yes ☐ No ☐
Wheelchair user? Yes ☐ No ☐	Assistance with booking required. Yes ☐ No ☐	
Transport required? Yes ☐ No ☐	Does the patient require a guide dog? Yes ☐ No ☐	

RELEVANT INFORMATION/REPORTS:

Duration of breathlessness symptoms: Free Text Prompt

Is breathlessness worsening over time? Free Text Prompt

Does the severity of the breathlessness vary significantly from day to day, or through the day? Free Text Prompt

Relevant past medical history: see below list

Allergies: see below list

Medication: see below list

CLINICAL HISTORY AND EXAMINATION FINDINGS:

Free Text Prompt

ADVISED INVESTIGATIONS PRIMARY CARE PRE-REFERRAL MANAGEMENT:

NT proBNP >2000 refer to rapid access heart failure.	If raised, please refer to heart failure service as per guidelines.
CXR performed within 3 months of referral	Please include the date of the CXR and location performed - assessor will review the film.
Spirometry +/- reversibility (depending on clinical context)	Not needed prior to referral, however, please include the report if available.
ECG performed within 3 months of referral	
FBC/TFTs/Renal function	

The practitioner clinically assessing this patient's referral may book the patient directly into a diagnostic/ investigation clinic based on the attached history and investigations. Tick this box if you feel this is inappropriate ☐

Please complete this section for all patients as the information is required to assess whether the patient is suitable for a remote consultation and 'direct to test'.

- ☐ Patient does not have dementia.
- ☐ Patient does not have a learning disability.
- ☐ Patient does not have a physical impairment that prevents them being ambulant from a wheelchair.
- ☐ Patient is suitable for telephone triage.

Frailty: Please enter the WHO Performance Score to establish if the patient is suitable to be triaged for 'direct to test'.

- ☐ 0. Fully active, able to carry on all pre-disease performance without restriction.
- ☐ 1. Restricted in physically strenuous activity but ambulatory and able to carry out work of a light or sedentary nature, e.g., light housework, office work.
- ☐ 2. Ambulatory and capable of all self-care but unable to carry out any work activities.
- ☐ 3. The patient is up and about more than 50% of waking hours.
- ☐ 4. Capable of only limited self-care; confined to bed or chair more than 50% of waking hours.
- ☐ 5. Completely disabled; cannot carry out any self-care. The patient is totally confined to bed or chair.

NHS No: NHS Number Full Name(inc. middle) (dob Date of Birth)

Problems

Medication

Allergies